3.3 Status Asthmaticus

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Cannot finish a sentence **Objective** • SpO_2 86% • Very little wheeze ("silent chest") • HR 130/min, RR 34/min • $PaCO_2$ rising on ABG	**ACTUAL** **Impaired gas exchange** related to severe bronchospasm and air trapping as evidenced by hypoxemia, silent chest and rising $PaCO_2$.	Within 4 hours, SpO_2 94% or more and ABG getting better.	• Call for urgent medical help. • Give high-flow oxygen and continuous nebulised bronchodilators as prescribed. • Give IV corticosteroid and IV magnesium sulfate as ordered. • Monitor SpO_2, HR, RR and level of consciousness continuously. • Get the intubation equipment ready.	SpO_2 on target and able to talk: **goal met**. If she becomes drowsy or exhausted, prepare for ICU and ventilation.
Subjective • "I am too tired to breathe." **Objective** • Using accessory muscles • Sweating heavily • Exhausted	**ACTUAL** **Fatigue** related to increased work of breathing as evidenced by exhaustion and accessory muscle use.	Within 4 hours she is working less hard to breathe.	• Keep her sitting up and well supported. • Do all her care for her; cut out any activity that is not needed. • Watch for signs that her breathing muscles are tiring.	Using accessory muscles less: **goal met**.
Subjective • *No specific complaint* **Objective** • On high-dose beta-agonists • On steroids	**RISK** **Risk for electrolyte imbalance** as evidenced by risk factors: high-dose bronchodilators (hypokalemia) and corticosteroids (hyperglycemia).	Potassium and glucose stay within the target range.	• Check serum potassium and blood glucose. • Watch the ECG for dysrhythmia. • Replace as ordered.	Values in range: **goal met**.